RFI

HEALTH TECHNOLOGY ECOSYSTEM

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NORIDIAN
HEALTHCARE SOLUTIONS

Elevating Operations, Enabling Care.

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Center for Medicare & Medicaid Services
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600 Pennsylvania Avenue NW, Suite CC-5610 (Annex J)
Washington, DC 20580

SUBMITTED BY:
Noridian Healthcare Solutions, LLC
4510 13th Avenue S., Suite 1
Fargo, ND 58103
Email: Tiffany.Dschaak@noridian.com
Telephone: (701) 433-3059

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HEALTH TECHNOLOGY ECOSYSTEM RFI

Response to Questions

C. Providers

1. Digital Health Apps

PR–1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

The Centers for Medicare & Medicaid Services (CMS) and its partners can take several steps to encourage providers—including those in rural and underserved areas—to adopt approved digital health products that enhance care delivery and operational efficiency. We recommend seeking guidance from Medicare Administrative Contractors (MACs) and industry experts currently managing the provider experience to identify, design, test, and implement more effective streamlined digital solutions. A multifaceted strategy may also include:

- **Implementing Customized Provider/Supplier Communication Plans:** Through our internet portal, we can regularly deliver electronic information to providers that offers a data visualization of their billing and coding trends compared to historical norms within their peer group. By alerting providers to emerging patterns and trends, this initiative does not increase provider burden yet promotes a sentinel effect, enabling proactive responses to potential provider issues before escalation.
- **Engaging Providers Early:** Involve providers in the development, testing, and implementation of digital health products to ensure the tools are relevant, user-friendly, and aligned with real-world clinical workflows. Provider input increases adoption and helps tailor solutions to local needs, including rural-specific challenges.
- **Streamlining Communication with Suppliers:** Develop mechanisms that enable easier, more direct communication between providers and suppliers, particularly for items like durable medical equipment. This can remove administrative barriers and expedite access for patients.
- **Aligning Financial Incentives:** Offer reimbursement incentives or performance-based bonuses to providers who successfully adopt and use approved digital tools. These incentives can help offset implementation costs and encourage widespread adoption.
- **Introducing Compliance Expectations:** Consider phased-in requirements or incentives for early adoption of approved products, where appropriate, to ensure consistent adoption across provider types while still allowing flexibility for resource-constrained practices.
- **Reducing Provider Burden:** Demonstrate how digital products can simplify day-to-day operations. For example:
  - Patient self-service access to health records can reduce staff time responding to routine inquiries.
  - Automated appointment reminders lower the burden of manual follow-up.
- **Improving the Provider Experience:** Show how digital tools can streamline workflows and improve communication. Examples include:

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  - Secure messaging platforms that provide flexibility and quicker interactions with patients.
  - Online check-ins that reduce time spent on intake and registration tasks and enable pre-visit copay collection.
- **Enhancing the Patient Experience:** Highlight the dual benefit to providers when patients have better access and control over their care. Features such as:
  - real-time access to medical records and test results,
  - convenient appointment scheduling,
  - digital billing and payment options,
  - and automated reminders for care management all contribute to improved patient satisfaction and reduced administrative follow-up for providers.

By combining industry thought leadership, provider engagement, operational support, and strategic incentives, CMS can foster greater adoption of digital health tools that benefit both providers and beneficiaries, especially in areas where resources are limited and care coordination is more challenging.

a. **What are the current obstacles?**

Several obstacles may discourage providers from adopting and promoting approved digital health products, despite the potential benefits for patient care and operational efficiency. These challenges fall into both practical and technical categories:

Access and Infrastructure Limitations

- **Lack of Reliable Internet:** Some rural providers operate in areas where broadband access is limited or unreliable, making it difficult to consistently access or utilize digital health applications.
- **Limited Availability of Digital Tools:** Some rural practices may not have access to the latest approved digital products, either due to vendor coverage gaps or lack of awareness.

Workforce and Operational Constraints

- **Financial Limitations:** Providers are already operating on razor-thin margins.
- **Staffing Challenges:** Rural areas often face workforce shortages, making it difficult for providers to allocate time or staff resources to learn, implement, or support new digital tools.
- **Training and Information Sharing Gaps:** Providers may lack access to adequate training or clear information on how to use tools effectively, especially when supporting patients who also need help navigating new technologies.
- **Implementation Costs:** Upfront investments in new systems such as hardware, licenses, integration, and training can be prohibitive for small or resource-constrained practices.

Technical Barriers

- **Multiple Systems of Record:** Providers frequently operate across fragmented Information Technology systems and the need to reconcile data from multiple sources can hinder adoption of a new digital tool.
- **No Dedicated Point of Contact:** When digital health vendors don't provide a clear point of contact or reliable support (e.g., unresponsive or confusing help desks), providers may abandon efforts to adopt the tool.

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- **System Reliability and Data Persistence Issues:** Concerns about downtime, data loss, or unreliable system performance can undermine provider trust in the tool.
- **Resistance to Change:** Both providers and patients may be hesitant to transition from familiar manual or paper-based processes to digital platforms, particularly if prior experiences have been frustrating or ineffective.

To support adoption in rural areas, it's essential to address these structural, financial, and technical obstacles. MACs play a crucial role in this effort by providing a feedback loop with recommendations to CMS and targeted outreach and education to health care providers. Contractors can empower providers to fully embrace digital health solutions by ensuring dependable access, offering hands-on support and training, reducing integration complexity, and fostering trust in the reliability of digital tools. Being actively involved in these conversations is vital as it allows contractors to understand and advocate for the specific needs and challenges faced by rural communities, thereby ensuring solutions are effective in diverse health care settings.

b. **What information should providers share with patients when using digital products in the provision of their care?**

Providers can enhance patient understanding, engagement, and outcomes by sharing clear, accessible information through digital health platforms. Examples of useful information to share with patients include:

- **Simplified Clinical Notes:** Providers can use artificial intelligence (AI) generated summaries to translate medical notes into layman's terms, helping patients better understand their condition, treatment plans, and next steps without being overwhelmed by complex medical language.
- **Referral Information:** Digital tools can be used to deliver timely, organized information about specialist referrals, including contact details, appointment instructions, and expectations for follow-up care.
- **Medication Instructions and Information:** Providers can share detailed medication guidance like dosing schedules and side effects to watch for, making it easier for patients to manage their prescriptions accurately.
- **Diagnosis Details in Plain Language:** AI tools can help break down complex diagnostic information into simple, patient-friendly language, empowering patients to understand their health status and participate more confidently in their care decisions.

Sharing this kind of information through digital platforms not only enhances transparency and trust, but also supports health literacy and strengthens the provider-patient relationship.

c. **What responsibilities do providers have when recommending use of a digital product by a patient?**

When recommending use of a digital health product, providers have a responsibility to ensure patients and their caregivers are informed, supported, and capable of engaging with the technology in a meaningful way. This responsibility is not limited to physicians alone; given staffing constraints and limited time, particularly in rural or underserved areas, nurses and health facility operational staff (such as patient access representatives) can play a key role. These team members can be trained to educate patients, address concerns, and assist with basic troubleshooting, helping bridge the gap between clinical care and technology use.

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Providers also have a duty to assess whether a digital health product is appropriate for each individual patient. This includes considering the patient's access to internet or compatible devices, as well as any disabilities or other barriers that may affect their ability to engage with the technology. An assessment tool or screening process could help providers identify patients who are good candidates for digital health tools and guide appropriate recommendations.

MACs can support these efforts through targeted outreach and education and by providing a feedback loop with recommendations to CMS. By providing resources and training tailored to provider needs, especially in underserved areas, MACs can reduce burden on health care staff and promote more equitable, effective use of digital health products.

PR–2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Past efforts like Provider Enrollment, Chain, and Ownership System (PECOS) 2.0 highlight key obstacles to successfully introducing new applications, such as prolonged development, limited user input, and technical inefficiencies. By learning from these challenges, we can take a more effective, user-centered approach to developing and deploying future solutions.

Obstacles

- **Multiple Programs Used Across Public Health Jurisdictions:**
  - **Obstacle:** Public health care programs (e.g., Medicare, Medicaid, Defense Health Agency, Veterans Affairs) each have unique technologies, thus hindering provider workflow efficiencies.
  - **Mitigation:** Using the same technology across public health care programs, such as through standardized application programming interfaces (APIs), offers numerous advantages by enhancing interoperability. This will improve health care quality, prevent medical errors, increase efficiency, reduce costs, hinder fraud, expand access to care, and ultimately, transform health outcomes for individuals and populations.
- **Prolonged Development Time and Lack of Usable Output:**
  - **Obstacle:** Projects like PECOS 2.0 faced significant delays and failed to produce usable results after extensive investment and missed deadlines.
  - **Mitigation:** Implement robust Agile project management practices to ensure timelines are realistic and achievable. Incorporate MACs and proven industry experts in the design, testing, and implementation to improve the provider experience and ensure satisfaction. Regularly review progress and adjust plans as needed to maintain momentum and deliver usable outputs incrementally.
- **Contract Cancellation and Financial Impact:**
  - **Obstacle:** CMS canceled contracts due to inefficiencies, leading to financial losses and setbacks.
  - **Mitigation:** Prioritize transparency in contract management. Clearly define expectations, milestones, and deliverables from the outset. Regularly evaluate contractor performance against these criteria to detect issues early and consider alternative contracting models if necessary.

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- **User Experience and Adoption Challenges:**
  - **Obstacle:** Existing systems like PECOS are criticized for being outdated and difficult to use, hindering adoption.
  - **Mitigation:** Involve administrators (i.e., MACs) and end-users (i.e., providers, suppliers) extensively throughout the development lifecycle. Use feedback systems to continuously gather user insights and preferences. Design solutions that address specific pain points and streamline workflows to enhance usability.
- **Technical Issues and Implementation Delays:**
  - **Obstacle:** Technical challenges are common, causing delays and impacting implementation timelines.
  - **Mitigation:** Partner with current administrators (i.e., MACs), leveraging their existing, proven implementation and provider engagement practices. Conduct thorough technical feasibility assessments before development begins. Maintain close collaboration between developers and technical experts to proactively identify and address potential issues. Implement agile development methodologies to iteratively test and refine solutions, using the support of MACs.

Lessons Learned

- **Effective Project Management:** Emphasize Agile project management with clear oversight and accountability.
- **Transparency and Communication:** Maintain open communication channels with end-users, providers, suppliers, and patients to manage expectations and foster trust.
- **User-Centric Design:** Prioritize user experience and usability throughout the development process.
- **Selecting Reliable Partners:** Choose development partners with a proven track record and capabilities aligned with project requirements.
- **Government-Industry Collaboration:** Foster closer collaboration between government agencies and MACs to leverage expertise and resources effectively.
- **Flexibility and Adaptability:** Build flexibility into project plans to accommodate evolving needs and technical challenges.

By learning from these challenges, we can take a more effective, user-centered approach to developing and deploying future solutions.

PR–3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

a. **Current challenges in accessing different data formats.**

Health care data exists in multiple formats: scanned documents, faxed records, lab results, free-text notes, and structured fields. Many electronic health record (EHR) systems are optimized only for structured data, making unstructured content like scanned referrals or physician notes difficult to access, search, or share electronically. This fragmentation can mean delayed or incomplete access to documentation needed to support coverage determinations, resulting in delays in claim payments or beneficiaries getting the necessary equipment.

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b. **Impact on patient care quality.**

Lack of full access to clinical data can compromise care:

- Delays and denials of prior authorizations can occur due to insufficient documentation.
- Delays in equipment delivery (e.g., wheelchairs, oxygen tanks) due to missing records negatively impact patient mobility and recovery. MACs have received supplier feedback indicating this is a significant pain point.
- Incomplete medical histories can lead to misinformed decisions, redundant testing, or inappropriate care plans.
- In rural areas, where provider options are limited and referrals often cross systems, inaccessible data impedes timely interventions and care continuity.

c. **Technical barriers to full data accessibility.**

- Legacy systems and siloed data architectures make integration across formats difficult.
- Unstructured data (e.g., free text, scanned Portable Document Formats) lacks standard metadata, making it hard to index, search, or extract meaning.
- Interoperability standards are not uniformly applied to all data types, limiting their effectiveness in real-world settings.
- Disparate vendor systems lack common formatting or APIs, especially in smaller or rural facilities.

d. **Cost or privacy implications of making all data formats interoperable.**

- Costs of converting and standardizing legacy or non-structured data can be high, especially for small practices or rural hospitals. Incentives could relieve providers of some cost.
- Privacy concerns increase when converting or extracting data from scanned documents or free text, as sensitive content may be inadvertently exposed.
- Strong safeguards must be implemented to comply with Health Insurance Portability and Accountability Act while enabling controlled, role-based access to all data formats.

e. **Priority level compared to other interoperability needs.**

While foundational interoperability issues (e.g., patient matching, standardized coding) remain important, enabling full access to all data formats is a high-priority for improving operational efficiency and patient outcomes. For stakeholders like durable medical equipment suppliers—who are enthusiastic about interoperability—this issue is not just technical but operationally urgent, as it directly affects their ability to respond to MAC documentation requests and submit timely claims.

Ensuring all data within an EHR system, regardless of format, is accessible and exchangeable is critical to advancing health care delivery and interoperability, particularly in urban and rural areas where care coordination and resource access vary widely. Overcoming technical and cost barriers must be a shared goal among providers, suppliers, policymakers, and technology vendors to realize the full promise of interoperability.

2. Data Exchange

PR–5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

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a. **Patient Access API.**
b. **Standardized API for Patient and Population Services.**
c. **Provider Directory API.**
d. **Provider Access API.**
e. **Payer-to-Payer API.**

Proposed Use Case: Enhancing MSP Coordination via Payer-to-Payer API
From a MAC perspective, the Payer-to-Payer API defined by the Health Level Seven Da Vinci workgroup presents a valuable opportunity to enhance coordination with the Benefits Coordination & Recovery Center (BCRC). While the BCRC is not a traditional payer, it plays a critical role in Medicare Secondary Payer (MSP) operations—identifying other health coverage, coordinating benefits, and recovering mistaken payments. By leveraging the Payer-to-Payer API, MACs could facilitate structured, consent-aware data exchanges that improve the accuracy and timeliness of MSP determinations. For example, the API's support for member match, provenance tracking, and secure data transfer could reduce misrouted correspondence, accelerate eligibility verification, and streamline recovery workflows. This would not only reduce administrative burden but also align with CMS's broader goals of interoperability, data liquidity, and patient empowerment.

f. **Prior Authorization API.**

Encouraging the adoption of the Prior Authorization API, as outlined in CMS Final Rule 0057-F, presents significant benefits for MACs, CMS, providers, and beneficiaries. By streamlining and automating the prior authorization process through Fast Healthcare Interoperability Resources (FHIR) based APIs, MACs can reduce administrative burden, improve operational efficiency, and enhance compliance with CMS interoperability mandates. Providers will benefit from faster, more transparent authorization decisions, enabling more timely care delivery and reducing delays in treatment. For beneficiaries, this translates into a more seamless health care experience with fewer disruptions in access to medically necessary services. CMS, in turn, gains improved data visibility and standardization across payers, supporting better oversight and policy development. Overall, the Prior Authorization API fosters a more connected, patient-centered health care ecosystem that aligns with CMS's broader goals of reducing burden and improving outcomes.

g. **Bulk FHIR—Do you support Group ID-based access filtering for population-specific queries?**
h. **SMART on FHIR—Do you support both EHR-launched and standalone app access? What does the process for application deployment entail?**
i. **CDS Hooks (for clinical decision support integrations).**

D. Payers

PA–2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

To support the successful implementation and utilization of FHIR APIs by MACs such as Noridian, CMS should consider a comprehensive support strategy that includes both technical and operational enablers. The following components are essential:

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- **Technical Reference Architecture:** A CMS-endorsed blueprint that defines the core components, standards, and integration patterns for secure and scalable FHIR API deployment.
- **GSA-Facilitated Software Procurement:** Collaboration between CMS and the General Services Administration (GSA) to negotiate discounted rates for FHIR server software and related tools, ensuring cost-effective access for MACs.
- **Funding for Software and Infrastructure:** Dedicated funding to support the acquisition, deployment, and maintenance of FHIR-compliant systems.
- **Implementation Toolkit and Best Practices:** A comprehensive guide tailored to MACs that includes step-by-step implementation instructions, case studies, and compliance checklists.
- **Technical Assistance and Troubleshooting Resources:** On-demand support to help MACs resolve implementation challenges and ensure smooth integration with existing systems.
- **Funding for Cross-Training and Staffing:** Financial support to offset the costs of training existing staff and hiring new personnel with the technical expertise required for FHIR API implementation.
- **MAC Workgroups and Knowledge Sharing Forums:** Regularly convened groups to facilitate peer learning, share implementation experiences, and align on standards and practices.

PA–5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

Payers and CMS industry partners, such as MACs, play a critical role in simplifying the clinical quality data responsibilities of providers, especially through collaboration on EHR technology advancements and the consolidation of interoperability and reporting requirements.

a. **How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?**

Payers, providers, and CMS industry partners have all shown strong interest in EHR capabilities that enable bulk extraction of clinical quality data from the source of truth. This approach allows payers to assume responsibility for quality calculations, relieving providers of the technical and administrative burden of managing complex quality reporting requirements themselves. As a next step, MACs will collaborate with payers, providers, and CMS to implement EHR-based bulk data extraction capabilities, enabling centralized quality calculations and reducing the reporting burden on providers.

- For payers, bulk extraction supports more accurate and timely quality measurement, improves risk adjustment accuracy, and enables stronger benchmarking for value-based payment models.
- For providers, bulk extraction reduces time spent on documentation, minimizes inconsistent requests for data submissions, and supports regulatory compliance with less manual effort. It also opens the door for timely insights into performance metrics that support clinical and operational improvements.
- For MACs, administrative efficiencies are realized, ultimately benefiting CMS.

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b. **In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?**

Streamlined processes will enhance MAC operations by improving accuracy and efficiency, bringing direct value to CMS.

Consolidating Interoperability and Reporting Requirements Across Payers and CMS

Currently, providers face fragmented reporting responsibilities across multiple payers and CMS, leading to duplicated work and inconsistent standards. Payers can help by:

- Aligning data standards and reporting frameworks across public and private sectors, enabling providers to invest in a single set of tools and workflows that serve both.
- Promoting use of shared, centralized data platforms (e.g., Arkansas SHARE) that allow for real-time exchange of quality data across systems and stakeholders.
- Supporting federated or centralized models where patient data is accessed once and used for multiple reporting and analytics purposes—reducing duplicative data entry and decreasing opportunities for error.
- Encouraging adoption of automated data feeds (e.g., from vital sign devices and labs) into EHRs to reduce manual entry and ensure consistency in reported metrics.

Technologies Supporting Real-Time Access and Streamlined Reporting

Payers can leverage emerging technologies to enhance both reporting efficiency and clinical care quality, including:

- FHIR-based APIs and bulk data exchange tools to automate the transfer of quality metrics from EHRs.
- Data aggregation platforms that consolidate inputs across multiple provider systems into a single reporting stream.
- Real-time dashboards that provide clinicians with actionable insights at the point of care—promoting informed decision-making while aligning with value-based care goals.
- Natural language processing and AI tools to extract meaningful data from free-text notes, scanned documents, and other unstructured formats, ensuring complete quality capture.

Simplifying quality reporting through payer-supported bulk data extraction and interoperability improvements is a win-win for both providers and payers. It reduces provider burden, enables more consistent and accurate quality measurement, and enhances patient care through better data availability. By aligning reporting requirements, leveraging automation, and investing in shared infrastructure, payers can help transform quality data collection from a regulatory burden into a meaningful tool for improving care and outcomes.

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